CLINICAL STUDY PROTOCOL

RELAPSE-STOP: A Phase III, Randomised, Double-Blind, Double-Dummy Study of Ovalitinib (MNS-2207), an Oral BTK Inhibitor, versus Teriflunomide in Adults with Relapsing Multiple Sclerosis

Protocol number: MNS-2207-301
Version and date: 2.0, 12 March 2026
IRAS ID: 1009871
Phase: Phase III
Sponsor: Meridian Neurosciences Ltd, Meridian House, 14 Kingsway Park, Cambridge CB4 0FZ
Contract research organisation: Northbridge CRO Ltd
Chief Investigator: Prof. Daniel Whitcombe

1. Synopsis

Population: adults aged 18 to 55 years with relapsing multiple sclerosis (2017 McDonald criteria), EDSS 0 to 5.5, and at least one relapse in the previous 12 months. Approximately 120 participants will be randomised at five UK sites (900 globally).

Design: randomised 1:1, double-blind, double-dummy, active-controlled. Ovalitinib 60 mg once daily versus Teriflunomide 14 mg once daily for 96 weeks.

Primary endpoint: annualised relapse rate up to Week 96. Key secondary endpoints: confirmed disability progression, new or enlarging T2 lesions on MRI, and NfL levels.

2. Eligibility (summary)

- Age 18 to 55 years at consent.
- Diagnosis of relapsing MS and EDSS 0 to 5.5 at screening.
- At least one documented relapse in the 12 months before screening.
- Exclusion: progressive MS; prior BTK inhibitor treatment; contraindication to MRI; pregnancy or breastfeeding.

3. Schedule of visits

Visits must take place within the windows below. Visits outside the window are recorded as protocol deviations.

Visit code	Visit	Timing	Window
SCR	Screening	Day -28 to Day -1	-
V1	Baseline / Randomisation (MRI)	Day 1	-
V2	Week 4	Day 29	+/- 3 days
V3	Week 12	Day 85	+/- 5 days
V4	Week 24 (MRI)	Day 169	+/- 7 days
V5	Week 36	Day 253	+/- 7 days
V6	Week 48 (MRI)	Day 337	+/- 7 days
V7	Week 60	Day 421	+/- 7 days
V8	Week 72 (MRI)	Day 505	+/- 7 days
V9	Week 84	Day 589	+/- 7 days
V10	Week 96 / End of Treatment (MRI)	Day 673	+/- 7 days
FU	Safety Follow-up	Week 100	+/- 7 days

On-site visits last approximately 2 to 3 hours. Visits that include an MRI scan last approximately 5 hours.

4. Safety reporting

Serious adverse events are reported to the sponsor within 24 hours of the site becoming aware. SUSARs are reported to the MHRA and the REC in line with UK regulations.

5. Participant payments and expenses

Participants receive a payment for each completed visit and reimbursement of reasonable travel expenses under the sponsor's Participant Expenses Policy. The clinical trial agreement with each site may set local rates. Claims are submitted through the CareCompanion app within 90 days of the visit and paid within 30 days.

- Visit payment: £40.00 for each completed on-site study visit (Screening to Safety Follow-up), paid by bank transfer.
- MRI visit supplement: an additional £25.00 for each visit that includes an MRI scan (Baseline, Weeks 24, 48, 72 and 96), in recognition of the longer visit.
- Mileage: £0.45 per mile for travel by private car, up to 150 miles per return journey. No receipt is required; the site confirms the distance.
- Public transport: the actual cost of standard-class rail, bus or tram fares, receipt or ticket required.
- Taxi: only where agreed in advance by the site (for example for participants with limited mobility), up to £60.00 per visit, receipt required.
- Car parking: actual cost up to £15.00 per visit, receipt required.
- Meals and refreshments: up to £12.00 per visit day for visits lasting more than 4 hours, receipt required.
- Overnight accommodation: up to £120.00 per night, maximum one night per visit, when the one-way journey is longer than 2.5 hours; must be approved by the site in advance; receipt required.
- Companion or carer: where a participant needs help to attend (for example an EDSS score of 6.0 or more), the travel and meal costs of one companion are reimbursed at the same rates.

6. Ethics and regulatory

The study will be conducted in accordance with ICH GCP (E6), the UK Policy Framework for Health and Social Care Research and the Medicines for Human Use (Clinical Trials) Regulations 2004. Favourable opinion from the Pennine Research Ethics Committee and HRA Approval are required before any site opens.